# 1

# Three Questions of Justice

## A FUNDAMENTAL QUESTION OF JUSTICE

As a matter of justice, what do we owe each other to promote and protect health in a population and to assist people when they are ill or disabled? I shall refer to this question about our social obligations as the "Fundamental Question" of justice for health.

My strategy for answering the Fundamental Question is to substitute for it three more specific "Focal Questions" (Daniels 2001):

1. Is health, and therefore health care and other factors that affect health, of special moral importance? To answer this question, we need to see how meeting health needs is connected with other goals of justice. Answering it may tell us whether we have obligations regarding health and its distribution within a population that do not apply to some other goods.
2. When are health inequalities unjust? To answer this question, we have to understand the factors and social policies that contribute to population health and health inequalities. The answer will help us understand which health inequalities it is most important to address.
3. How can we meet health needs fairly under resource constraints? Since health is not the only important good we pursue, resources are always limited. To answer this question, we must determine the source of our agreements and disagreements about priority setting. This will help guide policy under real-world conditions.

In this chapter, I shall explore each of these Focal Questions further, describe how the answers to them are related to each other, and explain how they fit together to form an answer to the Fundamental Question and thus provide us with a population view of justice and health. The Fundamental Question of justice, in the form of these three Focal Questions, underlies many controversial issues about health policy. My goal in this book is to



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show that appropriate answers to them offer useful guidance on these policy issues.

Before turning to the Focal Questions, I want to clarify the scope of the Fundamental Question and to warn against some inappropriate starting points in trying to answer it.

## Scope of the Fundamental Question

Perhaps the most common way to interpret the Fundamental Question is to view it in terms of medical interventions and services: What kinds of medical treatments should we include in public and private insurance plans? This interpretation is hardly surprising, since most of us believe that modern medicine is what sustains and restores our health. We have generally accepted the idea that modern medicine's magic bullets have made billions of people in the twentieth and twenty-first centuries live the longest and healthiest lives in human history. This focus on medicine and the inspiring myths that surround it is constantly reinforced in our culture by the mass media and by the professional and business interests that are dependent on the medical sector.

The Fundamental Question, however, has a much broader and more radical reach. We can do many other things to and for each other that have a great bearing, arguably more important than that of personal medical services, on the level of health in a population and the distribution of health status in it. For example, as individuals or corporations or communities, we may pollute the water and air, affecting the health of our community and those around it; or, as a society, we may pass and enforce strict environmental protection laws, making the necessary investment to provide all with clean water and air. As a society, we may leave the health and safety of workers in the workplace to the individual or collective bargaining market, or we may enact strict occupational health and safety standards and enforce them. As a community, we may limit access to firearms and require seat belts and airbags in cars, or we may adopt laissez-faire policies. As a society, we may actively educate the public about tobacco, safe sexual practices, and good diet, or we may not. The Fundamental Question asks what traditional public health policies we owe it to each other to pursue. Throughout this book, I will use the term "health care" to include both medical services and public health measures, since both are functionally aimed at individual and population health.

A central theme of this book is that the Fundamental Question reaches beyond health care broadly construed. The dramatic changes in life expectancy and population health status that began over a century ago in many developed countries predate the emergence of even the earliest successes of modern medical technology. Most scientists who have examined the significant rise in life expectancy and health status in much of the twentieth century attribute it to improved nutrition and housing – changes preceding the development of most vaccination programs and coming long before the

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emergence of antibiotics early in World War II. Broad social changes and intersectoral public health measures explain much of the improvement in our longer, healthier lives, more than does modern medicine.

Recent research and literature have focused attention on current health effects of social policies and practices well outside the health sector. As a society, we distribute important goods – such as education, housing, jobs, income, wealth, opportunity, political participation, and a sense of community – very unequally across subgroups that differ by race, ethnicity, gender, or class. These social and economic inequalities then produce health inequalities through complex mechanisms that we are just beginning to understand. We could, if we had the appropriate political understanding and will, limit these inequalities, or at least mitigate their effects through other social policies and practices. The Fundamental Question reaches out broadly to ask about all the "socially controllable factors" – medical care, broader forms of health care, public health measures, and the distribution of non-health-sector goods – that affect the level of population health as well as its distribution. Accordingly, an account of justice for health and health care should tell us how we ought to distribute all of these controllable factors.

The Fundamental Question includes the full range of socially controllable factors that affect health and its distribution. This has immediate implications for the way some philosophers have talked about health and what we owe each other. Rawls (1971), for example, suggests that health (like intelligence) is a "natural good," to be contrasted with social goods, such as health care or education. Understanding the breadth of the social factors that affect levels of population health and its distribution, however, undermines much of the force of this contrast. In whatever sense health is a natural good, its distribution is to a large extent socially determined, as is the aggregate level of health in a population. (The same point can be made about intelligence and cognitive skills more generally.)

A second contrast is even more misleading. Often disease and disability are viewed as natural misfortunes. Illness is just bad luck. But if something is merely unfortunate (as opposed to unfair), then issues of fairness or justice do not arise, or so some people claim.

Again, understanding the breadth of the social factors that affect health and its distribution undermines the view that we are concerned only with cosmic bad luck when we are sick or disabled. This is not to deny that bad luck is often involved. Even where the disadvantage imposed by illness or disability is not the *result* of socially controllable factors, we may still be able to *improve* the situation – by curing the condition or moderating its outcomes and its effects. An account of justice must explain what assistance we owe each other in meeting such needs, even when no one is responsible for making us needy. We should not allow misfortune to beget injustice.[1]

[1] Commentators as diverse as the libertarian Englehardt (1981) and the egalitarian Nagel (1997) have appealed to a contrast – or perhaps different contrasts – between what is

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Another aspect of the Fundamental Question needs clarification. The question asks about our obligations regarding overall levels of population health, not simply about health inequalities. This may seem surprising, since justice is sometimes described as an account of permissible inequalities. So characterized, it is necessarily concerned with the way health is distributed in a population. For example, a question of justice arises as soon as we observe that the health status of African Americans and some other minorities is worse than that of white non-minority Americans, even when we correct for the effects of differences in education, access to health insurance, and income. Similarly, when we observe that existing race differences in health status in the United States are dwarfed by class differences, further questions of equity – and thus justice – are raised. But it is not so obvious that an account of justice should also be concerned with the overall level of health in a population.[2]

Nevertheless, the Fundamental Question concerns the level of population health. Suppose that we encounter a society in which health status is (approximately) *equally poor* across its population subgroups. Suppose further that there are clear policies that would have improved the level of health in the population had they been implemented; indeed, suppose that they can still be implemented. The simple absence of health inequalities among races, ethnic groups, genders, or classes does not mean that there is no injustice here. After all, social policy has left them all worse off in health than they need be.

An account of justice for health and health care must address this issue. Failing to promote health in a population, that is, failing to promote normal functioning in it, fails to protect the opportunity or capability of people to function as free and equal citizens.[3] Failing to protect that opportunity or capability when we could reasonably do otherwise, I shall argue, is a failure to provide us with what we owe each other. It is unjust.

## Where Not to Begin

Faced with concerns about justice – for example, concerns about significant inequalities in health status or health services across races or classes – many people start by invoking the notion of individual or human rights. They assert that a violation of basic rights – a right to health or to health care – is

unfortunate and what is unfair. An account of justice must clarify what these contrasts are and what follows from them.

[2] My point is independent of how we measure population health, whether through narrow measures of infant mortality or life expectancy, or through more complex measures that combine length and quality of life, such as healthy life expectancy, disability-adjusted life years, or quality-adjusted life years, thus reflecting the societal burden of disease.

[3] My view is clearly in conflict with Walzer's (1983a, b) claim that a society that ignored health in favor of spiritual goals would be doing nothing unjust.

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involved. They believe that the just redesign of health-care institutions or other social structures can be done by appealing to such notions, for people can be galvanized into action when rights they cherish are violated.

Some people who appeal to these rights may assume that health care is more like certain social goods, say certain liberties, to which we assert right claims with clear conviction, than it is like other goods, such as computers or automobiles, to which we do not assert such claims (except as they may derive from property rights in general). Most people who assert a right to health or health care offer no particular theoretical account of its foundations – its grounds or justification – or its limits. They simply hope that if we all acknowledge such a right, we will unite behind the desired reform.

This direct, pragmatic appeal to rights faces significant objections. It does not carry us past our disagreements or uncertainties about the scope and limits of such right claims. It does not tell us what entitlements follow from these right claims. In addition, the pragmatism leaves us well short of determining which beliefs and theories count as an adequate justification for the rights claims. This failure is not just theoretical: It leaves us unable to resolve practical disputes about what these rights imply we owe each other.

My working assumption in this book is that the appeal to a right to health or to health care is not an appropriate starting point for an inquiry into just health or just health care. Rights are not moral fruits that spring up from bare earth, fully ripened, without cultivation. Rather, we may claim a right to health or health care only if it can be harvested from an acceptable general theory of distributive justice or from a more particular theory of justice for health and health care. Such a theory would tell us which kinds of right claims are legitimate. It would also help us specify the scope and limits of justified right claims. My working assumption borders on a philosophical commonplace, shared by many theoretical perspectives, despite the more general practice of the public, which often claims rights wherever strong interests are felt. To answer the difficult questions of what a right to health care or a right to health might include, we need a systematic theory of distributive justice for health-related needs. (I discuss the implications of my view for a right to health and health care in Chapter 5.)

Since we should not begin by appealing to a right to health or health care, should we instead look to general theories of justice that might clarify and justify such rights? Should we not just apply such theories to health and thus answer the Fundamental Question? This suggestion assumes that we can simply apply general theories of justice to the problem of health and health care. Unfortunately, in order to apply general theories to health and the socially controllable factors that affect it, we need to know what kind of a good health is and how its importance to us transfers to the socially controllable factors – medical, public health, and broader social determinants – that protect it. What looks like a natural way from within each theory to

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make such an application usually presupposes a particular view of the kind of good health or health care is.

One way to see the point is to ask whether health care services, say personal medical services, should be viewed as we view other "commodities" – things we agree to buy and sell in a market – in our society. Should we allow *inequalities* in the access to such services to vary with whatever economic inequalities are permissible according to more general principles of distributive justice? Or is health and therefore health care of special moral importance and not to be assimilated to other commodities, like cars or computers, whose distribution we allow to be governed by market exchanges among economic unequals? Put generally, this is one instance of the Focal Question "Is health or health care special?"

The difficulty can be overcome. The strategy I shall follow in this book is to begin with a more direct analysis of the kind of good that health is, and therefore the kind of importance we should give to health care and the other factors affecting population health and its distribution. Then I shall try to connect this account with more general principles of justice, modifying them if necessary to produce the most plausible fit. Rather than applying these general theories downward to examples of health and health care, I shall try to locate and connect with plausible principles of justice by building on the understanding we can arrive at about the kind of goods that health and health care are (see Chapter 2).

## THREE FOCAL QUESTIONS

All inquiry proceeds through questions. Every discipline develops tools and methods for answering the distinctive kinds of questions that define it. To find our way in the thicket of issues about justice and health, as in any area of inquiry, we must ask the right questions.

Each of the three Focal Questions identified earlier addresses a key aspect of the more general Fundamental Question of what we owe each other. If we can explain why societies do and should give special moral importance to meeting health needs, then we may be able to characterize the basis of our obligation to protect health. If we can explain when health inequalities are unjust, then we will have a better idea of what factors affecting population health and its distribution we are obliged to modify through social policy. If we know how to make fair and legitimate priority- and limit-setting decisions about meeting health needs under resource constraints, then we can guide our actions toward more just outcomes under nonideal conditions.

A reader of my earlier work, *Just Health Care*, might wonder why the central question in that book – is health and thus health care of special moral importance? – has now become three questions. Indeed, when I wrote *Just Health Care*, I thought that my answer to the first question sufficed to address the issues raised by the other two. I argued that health care was of special

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moral importance because protecting health protected opportunity. Since I also thought that health care – broadly construed to include medical services and traditional public health – was the main way to protect health, it seemed trivial to say when a health inequality is unjust. It is unjust when it is the result of unequal access to health care. Since I also thought that the impact on opportunity was the measure of the importance of meeting a health need, I thought I had addressed the question about meeting needs fairly. In short, I thought there was one answer to all key questions about justice and health.

Painful experience taught me that the three questions are distinct. When I learned that a broad range of social determinants of health account for much of the health inequality we see, I saw that the injustice of a health inequality between social groups could not generally be explained solely as the effect of unequal access to medical care or public health protections. When I realized that impact on opportunity could not function as the sole criterion for resource allocation, since there are "opportunity costs" to giving complete priority to those in greatest need and problems about how to aggregate benefits across different people, I saw that the resource-allocation question needed a more general answer. In fact, we could answer any two of the questions without being able to answer the third. It became clear to me that even if I was right about the special moral importance of health, my account failed to guide practice about which health inequalities need to be redressed or about the allocation of resources to meet health needs. In the spirit of learning from my mistakes, I want to motivate each of the three Focal Questions in what follows, liberating the first one as well from any misconceptions about the breadth of the determinants of health.

I do not claim and cannot prove that the Focal Questions are the only way, or even the best way, to organize our inquiry. What I do claim is that each question emerges from a common powerful observation and focuses on a puzzle central to concerns about justice. Each not only concerns a central set of theoretical issues, but also engages with policy issues relevant to them in a fruitful way. I shall first take up each Focal Question briefly to illustrate its centrality and fecundity. I shall then describe briefly the interrelationships of the three questions and suggest how they will fit together to answer the overarching question I began with.

## Is Health of Special Moral Importance?

This question was central to my inquiry in *Just Health Care* when I thought health care (broadly understood) was the controllable determinant of population health. It is no less central in *Just Health*, which adds broader social determinants to the socially controllable factors affecting population health and its distribution. The question remains central because the factors that affect health – whatever their scope – derive their moral importance from the moral importance of health.

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My focus on the question then and now is driven by the common observation that people who tolerate vast inequalities in wealth and power are often morally outraged when those who are ill cannot get care because they cannot pay for it. People who emphatically reject the general Marxist distributive principle, "From each according to his ability, to each according to his needs," embrace at least the second part of it when applied to health care. Not only individuals but also societies that tolerate – even glorify – significant inequalities in many goods treat health care differently, for they organize public and private cooperative schemes that distribute public health measures and medical services more equitably. Even the United States, the only developed country that fails to provide universal health insurance coverage, proves the rule. It provides universal Medicare coverage to its medically neediest group, the elderly, and publicly funded Medicaid to its very poorest citizens, if not to all in need. The insurance gap in the United States, which grew to 45 million people in 2006 from 33 million when President Bill Clinton attempted to introduce universal coverage in 1993, is widely condemned as unjust. Many poor developing countries have government health care systems that purport to provide universal health insurance coverage, in accord with publicly avowed principles and policy, though in practice they fall well short of doing so.

Are these seemingly schizophrenic attitudes toward social and economic inequality as opposed to health inequality incoherent or do they make moral sense? Why do people ascribe special moral importance to meeting health needs in their social practices? Should they assign it such moral importance? These questions remain central to an inquiry about justice and health even when we understand that the range of factors affecting health is wider than health care.

Many policy controversies relate to disagreements concerning how to answer these questions about the special importance of meeting health needs. The most familiar controversies involve access to medical services. For example, when Medicaid was introduced in the United States in 1965, the claim was made that it should and would avoid a two-class or two-tier medical system in which a distinct set of providers delivers services to the poor. A one-tier system seemed important because of the special importance of health care and because of the morally objectionable nature of inequalities in access to care. The fear of lower-quality health care derived from the widely held belief that "systems for the poor tend to be poor systems." The specific idea was that all physicians would be eligible to accept Medicaid reimbursement.

The outcome, however, proved quite different, in large part because political concessions were made to the medical profession. Physicians are not legally required to participate, and the low reimbursement rates for Medicaid services make these patients unattractive to most physicians. The resulting differentiation between the poor and the middle class – our two-tier

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reality – means that Medicaid makes an ambiguous statement at best about the special importance of health care.

Even universal coverage systems in other developed countries, which tend to include the poor with the broad middle class, differ about private supplementary insurance schemes that enable the best-off groups to receive faster or slightly better care. Canada and Norway, for example, prohibit the kind of supplementary scheme that the United Kingdom allows, indicating some disagreement about how far to carry the idea of the special importance of medical care. Despite these differences, all share the belief that meeting health care needs has special moral importance.

Private markets for medical insurance raise other questions about the special importance of health care. For example, Americans who buy individual health insurance policies, in contrast to employee-based group insurance policies, must undergo medical underwriting. Their health status and health risk are evaluated by private insurers, which then charge premiums reflecting the purchasers' levels of health risk. Similar underwriting practices are applied to life, property, and liability insurance. In all these cases, private insurers fear that people who are at higher risk and clearly need insurance will be the ones most likely to purchase it (this behavior is called "adverse selection" or "antiselection"). Thus, access to private medical insurance for individuals is not more special than access to other types of insurance, since the people who face the higher risks bear the higher costs. Many view the resulting inequality in access to insurance as unjust. Legislation in the United States in the mid-1990s attempted, with only marginal success, to reduce the exclusion of high-risk individuals from medical insurance.

Views about the special importance of health care affect policies for controlling rising medical costs. One common policy increases cost-sharing by those who use services. For example, Medicare has dramatically raised out-of-pocket expenditures for its patients, shifting the burden of payment to the sickest elderly (the recent Medicare drug benefit is a clumsy attempt to offset part of that problem). Health plans in the United States that once charged only token copayments for pharmacy benefits began, in the mid-1990s, to introduce substantial cost-sharing, either to motivate patients to choose drugs carefully or simply to reduce the proportion of costs borne by employers. Current national and corporate policies emphasize increased cost-sharing through "consumer-driven health plans" such as health savings accounts. These cost-sharing "experiments" (see Chapter 9; also Rosenthal and Daniels 2006) impose significant health risks on vulnerable subgroups. In general, the extensive use of cost-sharing suggests that many legislators, employers, and plan administrators believe that mechanisms for marketing other commodities are also appropriate for health care; that is, they believe that health care may not be as special as some think it should be.

Controversy about the special importance of medical care is not limited to the United States, of course. In the 1990s, many health systems in Europe

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experimented with market mechanisms, including efforts to stimulate competition among providers, as a way to improve efficiency. The more emphasis is placed on such mechanisms, the more we treat health care as a commodity like any other – possibly undercutting the view that it is of special importance. Indeed, because the introduction of some market measures threatened the special moral importance attributed to assuring equity in health care, these systems had to devise regulations allowing the government to ensure that equity was not threatened (Saltman 1995). This issue is particularly acute in many developing countries, where international agencies have encouraged privatization of many areas of health delivery and increased reliance on out-of-pocket health expenditures to reclaim health costs. In contrast to many of the European countries, these developing countries lack the powerful, reliable state apparatus to limit the tendency of market measures to produce inequality in access to services (see Chapter 9).

Controversy about the special importance of health care goes beyond the clinic, the hospital, and medical insurance to include public health policy. In the United States, for example, early efforts at environmental protection of workers were threatened by concerns that stringent measures were not worth the cost to employers or society. Employers sued the Environmental Protection Agency seeking relief from health and safety standards that were not "cost-beneficial." Only a crucial decision by the U.S. Supreme Court in 1978 left intact Congress's intent to protect health to the degree that it is "technologically feasible" to do so (see Chapter 7). Arguably, this standard reflects, at least in part, the judgment that protecting workers against workplace health risks is of special moral importance and worth the cost of developing new technologies to do so. In 2001, the Supreme Court upheld the authority of the federal government to legislate clean air standards that are more stringent than those some states would have enacted and more stringent than a direct calculation of costs and benefits, as preferred by businesses, might have authorized.

Is health, then, of special moral importance? What gives it that importance? Does that importance extend to the socially controllable factors that promote health?

Chapter 2 builds on the answer I gave to these questions in *Just Health Care*.[4] We keep people functioning normally by meeting their health needs, not just by providing health care but also by properly distributing the socially

[4] To support the view that health care is of special moral importance, some offer what I called the "Argument from Function" in *Just Health Care*. (a) The function of medical services is to meet medical needs. (b) The sole rational basis for distributing a good that meets certain needs is in proportion to those needs. (c) Therefore, the sole rational basis for distributing medical services is to meet medical needs. (d) Health status should determine access to medical needs.

This argument will not do as it stands. The *function* of lawn mowers is to meet lawn-mowing needs. No one insists, as a result, that willingness to pay for lawn mowers is the wrong basis for distributing them and that those who cannot afford them should be subsidized. Clearly,

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controllable factors that affect population health. The loss of function associated with disease and disability reduces the range of opportunities open to us compared to what it would be were we healthy or fully functional. By keeping people functioning normally, we protect their range of opportunities. If we have a social obligation to protect opportunity in this way, then we have a general framework for thinking about justice and health. One general theory of justice that justifies a principle protecting opportunity is Rawls's theory of justice as fairness (Rawls 1971, 2001). My claim about the relationship between health and opportunity thus provides a way of extending Rawls's theory to address the inequalities created by disease and disability, a key issue that Rawls had deliberately avoided. The extension vastly increases the power of his theory and its ability to respond to some critics. Other work on distributive justice that was developed after Rawls's initial work, and in part in response to it, also provides some support for the connection between health and opportunity and for the claim that we have some obligations of justice to protect opportunity.

In short, health is of special moral importance because it contributes to the range of opportunities open to us. Therefore, the socially controllable factors that promote health – medical services, traditional public health, and the distribution of the broader social determinants of health – derive special importance from their contribution to protecting opportunities. My move from *Just Health Care* to *Just Health* retains the core explanation of the connection between health and opportunity, but it expands the factors that derive importance from that connection because of their impact on population health and its distribution.

## When Are Health Inequalities Unjust?

Even if we know that health is of special moral importance because of its impact on opportunity, and even if we have an obligation to protect equality of opportunity, we do not know when health inequalities across social groups are unjust. If we think health care is the only way society has of protecting health, then, as I noted earlier, we might (mistakenly) infer that health inequalities are unjust when access to health care is unequal. The importance of this second Focal Question rests on understanding the many socially controllable factors that affect health inequalities. Answering the question will help us understand the breadth of what justice requires us to do to protect population health and its fair distribution.

The second Focal Question emerges from the common observation that health inequalities across social groups – by race or ethnic group, class, or

if the Argument from Function has any merit, it is only with regard to a special class of needs and things that function to meet them. We are back to a specific version of our first Focal Question, "Is health care special?"

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gender – occur in all societies. In every society, for example, we find a "social gradient" of health. If we graph life expectancy, a crude measure of health, or healthy life expectancy, a somewhat more precise measure of health, against socioeconomic status, we find that the richer (and, in the United States, the whiter) people are, the healthier and longer their lives. Different countries will have steeper or flatter gradients, depending on many facts about their distribution of other goods, such as education, income, wealth, housing, and access to health care. In the United States, when we control for inequalities in income, education, and insurance coverage, we still find significant racial inequalities in life expectancy and morbidity. Similar inequalities among racial and ethnic groups can be found in many countries. What is more, the class, race, and gender inequalities found in many countries cannot be explained simply by the presence or absence of poverty and deprivation; they persist well up the socioeconomic ladder and are significantly affected by bias associated with cultural and religious practices.

These inequalities persist even in developed countries that invest heavily in public health and assure universal coverage for medical services. Indeed, the presence of universal-coverage health systems has only a modest effect on health gradients, which are more sensitive to the distributions of other goods. Some of these health inequalities we may not know how to avoid or modify. Others, however, are clearly related to social policies that distribute many other important goods, such as education, income and wealth, and effective political participation. We may have various goals that lead to these social policies, and some of them may be required by other considerations of justice.

Are all of these commonly observed health inequalities unjust? Should we view some of them as acceptable trade-offs against other things we also value highly in society? Although our answer to the first Focal Question suggests that health and health care are of special moral importance because of their effects on opportunity, opportunity is not the only thing we value. However important, it is not the only good of special moral importance, even from the perspective of justice. In short, the second Focal Question is important because it reminds us that what justice requires in policy that affects health and the acceptability of health inequalities must be compatible with what justice requires in the distribution of a broad range of social goods. The fact that health is not simply the product of health care means that we cannot easily isolate health from broader social justice (as I thought we could in *Just Health Care*).

The second Focal Question underlies many important policy controversies. Often it is claimed that we must accept some, perhaps a great deal of, inequality in order to stimulate economic development in developing countries or more rapid growth in any country. If, to promote development or growth, we must accept significant inequalities, then we may be inducing health inequalities. There is much controversy in developing countries

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about the consequences of different approaches to acceptable socioeconomic inequalities. For example, some people claim that we can reduce poverty most rapidly by accepting significant inequalities; as a result, countries may reduce health inequalities more than they would if they accepted a slower rate of growth. Others insist that policies improve population health more if they aim directly at producing a more equal distribution of health, even if this means a slower rate of growth. In developed countries, there are similar controversies about the consequences of different degrees of economic inequality. Some argue that jobs are created more quickly if we allow greater economic inequality and provide a narrower social safety net; others say that a broader social safety net and the redistribution it involves make the effect of unemployment more bearable and reduce health inequalities more. These controversies, I suggest, are not just about *empirical* uncertainty. They also reflect underlying moral disagreements about what kinds of inequality, and what trade-offs among them, are morally acceptable or fair.

Chapter 3 offers an approach to answering the second Focal Question. To answer it, we must not only understand the socially controllable factors that affect population health and its distribution, but also have an account of when the distribution of these factors is fair or just. As I noted, for example, it is not enough simply to say that poverty is unjust and whatever health inequality follows from dire poverty is therefore unjust. Instead, we must have an account of when income inequalities are fair or just – despite or including their effects on health.

My approach to this question is based on a striking observation: The general principles of justice that Rawls argues for in his theory of justice as fairness capture the key social determinants of health, especially if one includes, as I do, health care in the institutions that protect opportunity. If Rawls's principles constitute a fair distribution of these socially controllable factors affecting health, then we may have made some headway in deciding which health inequalities are unjust. Using this approach, one might say that the inequalities in health that remain after a fair distribution of other goods should count as acceptable or fair inequalities. Others, however, may read the priority Rawls ascribes to opportunity as requiring a further reduction in health inequalities. Despite this controversy about remaining health inequalities, the appeal to Rawls to illustrate what a fair distribution of determinants of health might look like is a distinct advance over the simpler extension of his theory provided by my answer to the first Focal Question.

In effect, my answer to the second Focal Question suggests that justice as fairness is good for our health. It arguably leads to a fair distribution of population health, and it does so in a way that social justice in general endorses. This answer is particularly startling since Rawls simplified the development of his theory by postulating that all people function normally over a normal lifespan, barring the prospect of disease and disability from his social

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contract. What is startling is that a theory that develops an account of social and political well-being turns out to generalize into a theory that provides for our physical and mental health as well. The answer is, however, only a partial answer, and I shall examine its adequacy.

## How Can We Meet Health Needs Fairly under Resource Limits?

Even if we have acceptable answers to the first two Focal Questions, we will face further challenges presented by the third: How can we meet health needs fairly under reasonable resource limits? Suppose that we agree about how much moral importance we should attribute to meeting health needs, because we understand the connection between health and opportunity. Suppose that we also agree about which health inequalities we find unjust, and suppose that we even agree on what we consider reasonable resource limits on efforts to meet health needs. Unfortunately, all this agreement still does not tell us how to meet conflicting health care needs fairly when we cannot meet them all. We observe that all societies face this problem, even when they have a social consensus on the importance of health and a good grasp of the measures that may affect the distribution of population health. Although an answer to the third Focal Question is not provided by answers to the other two Focal Questions, the answer must still be compatible with these other answers.

The problem raised by the third Focal Question arises because we lack a consensus on principles for resolving conflicting claims on resources for meeting health needs.[5] For example, Rawls's principle assuring fair equality of opportunity as a basis for the social obligation to promote health is too general and indeterminate to help us make specific resource-allocation decisions. It might incline us to favor giving greater priority to those whose health is worse off, since their opportunities are more seriously reduced. But few would accept giving complete priority to those who are worst off, especially if that meant not meeting the health needs of others who have significant, but less serious, illnesses. Different people would make these tradeoffs in different ways. We lack a principled account of how they should be made.

Other familiar areas of reasonable moral disagreement are revealed when a public or private insurer denies coverage for a "last-chance" therapy that is not fully proven but that offers an individual her only hope. Reasonable people may disagree about how much weight to give to stewardship of scarce resources and how much to compassionate use of these resources for specific individuals. Again, we lack agreement on principles that can resolve this dispute.

[5] Actually, the problem is general (Daniels 1993) and arises for claims about other needs as well, but I focus here on health needs.

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Any decision we make about how to use resources to meet health needs can create winners and losers. Yet, we lack a consensus on principles for resolving disputes among these winners and losers. We have reasonable moral disagreement about how to treat people fairly. Perhaps we can eventually arrive at philosophical analyses of these issues that produce wide acceptance of appropriate principles, and we should invest in such work, but in the real world, resources must be allocated in real time in a way that is perceived to be legitimate and fair.

Chapter 4, drawing on my work with Jim Sabin (Daniels and Sabin 2002), proposes that we rely on a fair deliberative process to reduce disagreements about resource allocation. This is an appeal to procedural justice: The outcomes of fair procedures should count as fair when we cannot agree on principles for resolving the disputes. The specific conditions such a process should satisfy – publicity of rationales, a search for relevant reasons that are properly vetted by those affected by the decision, opportunity for revising decisions in light of new evidence and arguments, and assurance that these conditions are uniformly enforced – make decision makers accountable for the reasonableness of their decisions. They provide the conditions for the legitimacy of controversial decisions. At the same time, this fair process should be limited by the considerations of justice that emerge from our answers to the first and second Focal Questions. A complete account of just health and just health care must appeal both to principles and to process, and the two components must cohere appropriately.

Like the first two Focal Questions, the third one underlies policy controversy in addition to posing a theoretical puzzle. For example, the policy problem of access to health care breaks down into two questions: Who has access? To what do they have access? The latter question has become increasingly central to policy discussions in the United States and elsewhere. Since medical services today deliver vastly more expensive, varied, and effective products than they did years ago, it has become crucial to limit care. At whatever level of funding a society establishes as reasonable for it – from 6 percent in the United Kingdom to over 14 percent in the United States, it is still not possible to meet all competing health needs. Limits must be set.

Controversy focuses on what limits are fair and how they can be set in a fair way. One volatile form of this controversy is the frequent protest against denials of care, especially when people are desperately ill and all proven medical remedies have failed. In Cambridge, England, great media controversy surrounded the Cambridge Health Authority's denial of an experimental treatment to Child B (Ham and Pickard 1998). In Boston, Massachusetts, similar media controversy focused on Medicaid denial of an unproven therapy for a dying child. In both cases, private charitable donations were used to obtain the (unsuccessful) treatments. In the early 1990s in the United States, litigation and legislation made bone marrow transplants the standard of care for treating advanced breast cancer, delaying the results of

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clinical trials that later showed that these treatments actually reduced life expectancy.

It is not just last-chance therapies that are the focus of limit setting. Most cost reduction in the United States throughout the 1980s resulted from limits on hospital stays for which insurers (especially Medicare) would reimburse hospitals. (By squeezing hard on length of hospital stay, payers produced a bulge elsewhere in the balloon of medical costs, since new technologies were encouraged that led to the costly delivery at home of services previously available only in hospitals.) Since the mid-1990s, drug costs in the United States (and elsewhere) have been rising at a rate of over 10 percent per year, making them the fastest-growing part of the health care budget. The private for-profit companies that manage the drug benefits for most insured Americans have adopted various strategies for limit setting. Yet, the establishment of limited formularies and various types of copayment has produced much discontent among patients and physicians. It has also produced much confusion in the implementation of the Medicare Part D drug benefit.

During the 1990s in the United States, physician reimbursements were restructured from the common fee-for-service arrangements of previous decades to various types of income withholding and capitatiou. These new arrangements give physicians incentives to reduce utilization, but they also increase distrust. Patients fear that they will be undertreated by physicians who try to increase earnings. (Apparently, the fear of undertreatment is greater than the fear of overtreatment, which was the clear risk of fee-for-service incentives.)

Supplementing a principled account of justice and health with an appeal to fair process where principles do not resolve disputes is a strategy that many philosophers find problematic. Many resist the idea that philosophical investigation cannot produce a consensus on the right view; there is considerable distrust of a fair process that is open to abuse and misuse and can end up "merely political" in the worst sense. Nevertheless, the approach I describe has wide appeal in practical settings where the challenge of reasonable disagreement must be met.

## A POPULATION VIEW OF JUSTICE AND HEALTH

Answers to the three Focal Questions cohere to provide an answer to the Fundamental Question about what we owe each other in promoting our health and assisting each other when we face illness or disability. Together the three answers offer a population view of justice and health. Chapters 2–4 develop these answers in greater detail, and Chapter 5 describes some of their central implications. Before embarking on those more detailed tasks, I want to make sure that the big picture remains clear. I need to say more about how the answers fit together to form an integrated population view.

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My answer to the first Focal Question shows that the special moral importance of health derives from its impact on our opportunities. Thus, it explains why protecting health is a social obligation, not simply one on individuals with particular roles, such as physicians. As members of a society seeking fair terms of cooperation to protect each other's health, we owe it to each other to design institutions that do that and create a collective space to protect opportunity in this way. This claim about the importance of protecting opportunity is supported by many general theories of justice (though obviously not all).

My answer to the second Focal Question is that a health inequality is unjust when it derives from an unjust distribution of the socially controllable factors affecting population health and its distribution. I illustrated what is meant by a just distribution of those determinants of health by noting how Rawls's principles of justice as fairness would distribute them. This answer explains why and how all groups are included in the collective obligation to promote health, why none should be left out or neglected, and how we can tell which groups are being shortchanged. To answer this Focal Question, we must identify the sources of health inequality in populations and learn how to correct them.

Despite the principled account that emerges from these first two answers, a problem remains. The principles are too indeterminate to tell us how to resolve disputes among those with legitimate claims on resources for meeting health needs. Reasonable people disagree about how those claims should be addressed, despite the general guidance provided by the answers to the first two Focal Questions. My answer to the third Focal Question shows that we must supplement guidance from general principles with a fair deliberative process. Only in this way can the choices that unavoidably create winners and losers among reasonable claimants on assistance be viewed as legitimate.

In short, my answers to the three Focal Questions address the central issues involved in justice and population health: We understand the basis on which we owe people assistance in protecting health. We can tell when some people or groups are being treated less fairly than others, and we see more clearly what we should do about it. We understand how to arrive at fair and legitimate decisions about competing health needs when we cannot meet them all.

These answers limit each other in important ways. The appeal to fair process is intended to resolve disputes about how best to protect the opportunities of groups with competing claims on health resources. Relying on fair process to set limits and priorities for resource allocation does not replace broader obligations of justice to protect opportunity and distribute other goods fairly; rather, it is limited by those obligations. It is a way to produce agreement about what those obligations mean in a world of competing claims. Similarly, answers to the first two Focal Questions limit each other.

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Thus, the connection between health and opportunity has a bearing on when health inequalities between groups are unjust: Health inequalities that are unjust must have something to do with unjustifiable inequalities in the resulting opportunities for people. Conversely, these inequalities in opportunity become unjustifiable, depending on the rationale for the distribution of the determinants of health.

Integrating these answers thus gives us a population view of just health. Together they tell us how a society should organize itself to address its health needs equitably. This focus on equity must nevertheless be compatible with other concerns of social justice, which I pursue in Part II. Moreover, a population view of health must be global, some aspects of which I consider in Part III.

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